Case study: Frank

Condition: **Spinal cord injury – C4 AIS D incomplete tetraplegia (central cord pattern)**

77-year-old man · Acute orthopaedic and spinal ward, public hospital

Frank Doyle is a 77-year-old man with an **incomplete cervical spinal cord injury (C4 AIS D) with a central cord pattern**, sustained three weeks ago when he fell backwards down two steps at home. He is being managed without surgery in a rigid cervical collar, which the spinal team has documented must be worn at all times for six weeks. Frank lives in a lowset house in Bundaberg with his wife, June (75), who has Parkinson's disease. He has been her main carer for several years. Frank's stepdaughter, Kerry, lives nearby and has been looking after June since the fall.

Physiotherapy has documented that Frank walks around the ward with a four-wheeled walker and supervision, and climbs two steps with a rail. Nursing notes describe him as "pleasant, cooperative and independent with mobility". The medical team plans to discharge him home tomorrow, and the bed manager has booked patient transport for 10 am. The occupational therapy referral was only received yesterday afternoon.

You have been asked to complete an occupational therapy breakfast and self-care assessment before discharge. When you arrive at 8 am, Frank is sitting on the edge of his bed, shaving with an electric razor. His cervical collar is lying on the overbed table. He smiles and says, "Morning! I just take it off for a shave and a wash. It itches like mad." When you ask how often he does this, he shrugs. "Every morning, and after my shower. I'm careful. The nurses are busy, no need to bother them."

Frank chats easily about his lawn bowls club and says he plans to be "back on the green by Christmas". When his breakfast arrives, however, he cannot peel the lid off his yoghurt or open the blister pack of his morning tablets, and he asks you to cut his toast. He tries to do up the buttons on his pyjama top but gives up after several attempts. He shakes out his hands and tells you they feel "like they're burning, all the time", especially at night. "My legs are fine. The hands will come back."

When you ask about his routine at home, Frank says he does "everything" for June. He manages her tablets, which have to be given at set times, cooks the meals, helps her shower and drives her to appointments. He tells you, laughing, that the nurses have "had to help with the embarrassing bits" after he goes to the toilet. "That won't be a problem at home."

There is no documentation of whether the nursing or medical team know that Frank has been removing his collar, and no formal assessment of his hand function has been recorded. There is no clear information about June's current needs, whether the couple receives any community services, or how June has been managing her medications since Frank's fall.

As you finish breakfast, the nurse unit manager stops at the bedside and asks whether OT can complete the discharge paperwork today, as transport is confirmed for tomorrow morning. Frank's phone rings and he puts Kerry on speaker. She says, "Mum's had a terrible week, and I'm back at work Monday. He's walking, isn't he? He just needs to get home." Frank reaches for his collar and starts trying to put it back on himself.

You are the occupational therapy student completing a pre-discharge breakfast and self-care assessment with Frank and your clinical supervisor is not currently in the room.

Present Frank's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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